Comprehensive Clinical Health Assessment Questions

18 Years Diagnostics Industry Perspective • 3R Methodology (Restoration, Regulation, Resilience)

This official document provides the complete diagnostic architecture and question bank utilized across the **HealthwithReshmi™ Functional Health Portal**. It includes both the **Interactive Health Assessment (Part 1)** — designed to decode gut permeability, CO■ tolerance, and metabolic rhythms — and the **Pre-Consultation Clinical Intake Questionnaire (Part 2)** — used by Reshmi Verma to formulate tailored, root-cause recovery protocols.

PART 1: INTERACTIVE FUNCTIONAL HEALTH ASSESSMENT (9 CLINICAL DOMAINS)

SECTION 1 — GUT MUCOSAL & DIGESTIVE HEALTH

Decoding intestinal permeability (leaky gut), microbiome dysbiosis, and stomach acid kinetics.

Q1. When do you typically experience bloating, gas, or distension?

Clinical Significance: Timing of post-meal bloating reveals the struggling anatomical section.

Diagnostic Response Option	Biomarker Signal / Diagnostic Marker	Score
• Severe distension within 30–45 mins of eating	**Hypochlorhydria / Low Stomach Acid**	**35 / 100**
• Lower abdominal bloating late evening or 2–3 hours after meals	**Microbiome Fermentation / SIBO**	**45 / 100**
• Frequent acid reflux, heartburn, or persistent chest heaviness after meals	**Gastroesophageal Reflux / LES Laxity**	**40 / 100**
• No visible distension, but chronic post-meal sluggishness and slow digestion	**Sluggish Motility / Bile Stasis**	**65 / 100**
• Flat stomach, effortless digestion with zero post-meal discomfort	**Optimal Digestive Ease**	**95 / 100**

Q2. How would you classify your bowel motility & stool consistency?

Clinical Significance: Elimination is the primary exit pathway for metabolized hormones and cellular toxins.

Diagnostic Response Option	Biomarker Signal / Diagnostic Marker	Score
• Hard, pellet-like stools or skipping bowel movements for 1–2 days	**Constipation & Toxin Reabsorption**	**35 / 100**
• Urgent, loose, or undigested food particles in stool	**Rapid Transit / Malabsorption**	**40 / 100**
• Alternating between constipation and loose stools	**Enteric Nervous Irritability (IBS-M)**	**45 / 100**
• Daily smooth, well-formed, effortless bowel movement (Bristol Type 4)	**Balanced Intestinal Motility**	**95 / 100**

Q3. Do you experience food sensitivities or inflammatory reactions?

Clinical Significance: Intestinal permeability (Leaky Gut) allows macromolecular antigens into systemic circulation.

Diagnostic Response Option	Biomarker Signal / Diagnostic Marker	Score
• React strongly to gluten, dairy, high-FODMAPs, or oily restaurant meals	**Severe Leaky Gut / Endotoxemia**	**30 / 100**
• Brain fog, joint aches, or skin flare-ups/acne within 24 hours of certain foods	**Immune Complex Flare**	**45 / 100**
• Mild occasional discomfort, but can generally tolerate most whole foods	**Moderate Tolerance**	**70 / 100**
• Can digest diverse whole, fermented, and fibrous foods with zero systemic flare-ups	**High Immunological Tolerance**	**95 / 100**

SECTION 2 — AUTONOMIC BREATH & VAGAL TONE

Measuring CO■ tolerance, diaphragmatic mechanics, and parasympathetic vagal recovery.

Q4. What is your involuntary daytime breathing pattern?

Clinical Significance: Over-breathing depletes CO■, suppressing Bohr Effect oxygen diffusion to tissues.

Diagnostic Response Option	Biomarker Signal / Diagnostic Marker	Score
• Frequent upper-chest breathing, sighing, yawning, or feeling of 'air hunger'	**Chronic Hyperventilation**	**35 / 100**
• Mouth breathing during focused desk work, speech, or light physical exertion	**Airway Collapse & Mouth Breathing**	**45 / 100**
• Nasal breathing mostly, but chest moves instead of diaphragm when stressed	**Dysfunctional Biomechanics**	**65 / 100**
• Silent, calm, diaphragmatic nasal breathing (Inhale & exhale through nose 24/7)	**Optimal Respiration Mechanics**	**95 / 100**

Q5. How does your nervous system respond to acute daily stress?

Clinical Significance: The vagus nerve modulates heart rate variability (HRV), bile flow, and somatic calmness.

Diagnostic Response Option	Biomarker Signal / Diagnostic Marker	Score
• Constant sympathetic overdrive: Racing heart, muscle tightness, anxiety surges	**Low Vagal Tone / Sympathetic Overdrive**	**30 / 100**
• Afternoon energy slump paired with agitation, irritability, or brain fog	**HPA-Axis Instability**	**50 / 100**
• Experience work stress, but can unwind after a few hours with conscious effort	**Moderate Nervous Regulation**	**70 / 100**
• Resilient vagal brake: Quick emotional reset, calm pulse, high somatic composure	**Elite Vagal Tone**	**95 / 100**

Q6. How is your physical stamina and aerobic breath-hold capacity?

Clinical Significance: Tolerance to carbon dioxide (BOLT score) is the physiological bottleneck of aerobic energy.

Diagnostic Response Option	Biomarker Signal / Diagnostic Marker	Score
• Get breathless climbing 1–2 flights of stairs; breath-hold capacity under 15s	**Low CO■ Tolerance**	**35 / 100**
• Moderate stamina, but experience intense physical exhaustion after normal workout	**Mitochondrial Aerobic Deficit**	**55 / 100**
• Decent stamina during exercise, but recovery takes 24–48 hours	**Moderate Aerobic Buffer**	**70 / 100**
• High aerobic stamina, easy nasal workouts, effortless recovery, BOLT score 30s+	**Optimal Cellular Respiration**	**95 / 100**

SECTION 3 — CIRCADIAN RHYTHM, HORMONES & CELLULAR ENERGY

Analyzing cortisol rhythms, thyroid/insulin sensitivity, and deep restorative sleep architecture.

Q7. How is your nighttime sleep architecture and morning wakefulness?

Clinical Significance: Fragmented sleep spikes morning fasting insulin and suppresses glymphatic brain detox.

Diagnostic Response Option	Biomarker Signal / Diagnostic Marker	Score
• Fragmented sleep, frequent waking around 2–4 AM, or waking up with dry mouth	**Circadian Disruption / Airway Instability**	**30 / 100**
• Take over 45 minutes to fall asleep due to racing thoughts; dependent on screens	**Delayed Melatonin Phase**	**45 / 100**
• Sleep 7–8 hours, but wake up feeling heavy, sluggish, and unrefreshed without caffeine	**Mitochondrial Sleep Deficit**	**60 / 100**
• Deep, restorative, uninterrupted sleep; wake up refreshed before alarm	**Synchronized Circadian Rhythm**	**95 / 100**

Q8. Which metabolic or hormonal pattern resonates most with you?

Clinical Significance: Insulin, cortisol, and thyroid dictate cellular energy partition and fat storage.

Diagnostic Response Option	Biomarker Signal / Diagnostic Marker	Score
• Stubborn belly fat, intense post-meal sweet cravings, or shaky if meals delayed	**Insulin Resistance / Hypoglycemia**	**35 / 100**
• Cold hands/feet, hair thinning, sluggish metabolism, or irregular cycles / PCOS	**Thyroid & Endocrine Imbalance**	**40 / 100**
• Energy drops drastically between 3 PM – 5 PM, but otherwise manageable	**Cortisol Rhythm Dip**	**65 / 100**
• Stable all-day cellular energy, balanced body composition, zero urgent sugar cravings	**High Metabolic Plasticity**	**95 / 100**

Q9. How long have you been navigating these health roadblocks?

Clinical Significance: Calibrates the duration required for therapeutic cellular repair and habit stack integration.

Diagnostic Response Option	Biomarker Signal / Diagnostic Marker	Score
• Less than 6 months (Acute onset)	**Acute Phase Response**	**90 / 100**
• 6 months to 2 years (Establishing multi-system compensation pattern)	**Sub-Chronic Phase**	**70 / 100**
• 2 to 5 years (Deeply ingrained cellular dysfunction and systemic adaptation)	**Chronic Phase**	**50 / 100**
• More than 5 years (Multi-decade metabolic burnout — requires phased roadmap)	**Deep Chronic Adaptation**	**35 / 100**

PART 2: CLINICAL PRE-CONSULTATION INTAKE QUESTIONNAIRE (SECTIONS A TO F)

SECTION A — PERSONAL & DEMOGRAPHIC INFORMATION

Confidential patient profile for clinical record keeping and consult scheduling.

Q#	Clinical Field / Question	Format / Options	Clinical Objective
Q1	**Full Legal Name**	Text input (e.g. Patient Full Name)	*Mandatory identity verification*
Q2	**Email Address**	Email input	*Secure telehealth link & report delivery*
Q3	**Contact / WhatsApp Number**	Phone with country code	*Direct clinical coordinator channel*
Q4	**Age (Years)**	Numeric	*Metabolic baseline calibration*
Q5	**Biological Gender**	Female / Male / Other	*Endocrine and cycle reference metrics*
Q6	**City & Country**	Text input	*Timezone scheduling & food availability*
Q7	**Occupation / Work Style**	Text input	*Ergonomic, screen-time & stress exposure*

SECTION B — YOUR MAIN HEALTH CONCERN

Help us decode your primary health complaints, duration, and root-cause trajectory.

Q#	Clinical Field / Question	Format / Options	Clinical Objective
Q10	**Primary Health Concern**	Select: Gut/IBS/SIBO, PCOS/Hormones, Thyroid, Weight Plateau, Insulin/Diabetes, Chronic Fatigue, Breathing/Sleep, Skin/Inflammation, Athletic VO2 Max, Longevity	*Guides diagnostic panel selection*
Q11	**Concern Description**	Detailed narrative in patient's words	*Symptom triggers, daily frequency & sensations*
Q12	**Concern Duration**	Less than 1 mo / 1–6 mos / 6–12 mos / > 1 year	*Differentiates acute vs chronic compensation*
Q13	**Daily Life Impact**	Open response	*Evaluates functional disability & productivity cost*
Q14	**Consulted Medical Professional?**	Yes / No	*Identifies prior clinical workups*
Q15	**Prior Treatments & Outcomes**	Open response (diets, therapies, meds tried)	*Prevents repeating failed therapeutic strategies*

SECTION C — HEALTH & MEDICAL HISTORY

Medical diagnoses, prescription medications, surgeries, and biological history.

Q#	Clinical Field / Question	Format / Options	Clinical Objective
Q16	**Diagnosed Medical Conditions**	Open response (Thyroid, Fatty Liver, GERD, etc.)	*Screens for underlying systemic pathologies*
Q17	**Current Medications & Supplements**	List names, dosages & frequencies	*Checks drug-nutrient depletions & interactions*
Q18	**Known Food Allergies & Intolerances**	Open response (Dairy, Gluten, Nuts, Histamines)	*Informs elimination & gut mucosal rebuild protocols*
Q19	**Surgeries & Medical Events**	Open response (Gallbladder, C-section, Appendectomy)	*Assesses organ function & enteric nerve integrity*
Q20	**Laboratory Blood Reports**	File upload (CBC, Thyroid, Lipids, HbA1c, Vit D)	*Objective biomarker validation (We measure, never guess)*
Q21	**Additional Medical Context**	Family history, genetic predisposition, notes	*Full biological pedigree mapping*

SECTION D — BODY & LIFESTYLE PROFILE

Physical measurements, movement metrics, sleep architecture, and nervous system pacing.

Q#	Clinical Field / Question	Format / Options	Clinical Objective
Q22	**Height (cm)**	Numeric	*BMI, visceral adiposity & BMR calculation*
Q23	**Current Weight (kg)**	Numeric	*Metabolic baseline*
Q24	**Weight Changes (Last 3–6 Months)**	Gained / Stable / Lost	*Detects metabolic slowdown or catabolism*
Q25	**Physical Activity Level**	Sedentary / Lightly Active / Moderate / Very Active	*Mitochondrial turnover & carb threshold*
Q26	**Sleep Duration & Breathing Patterns**	Hours, night waking, snoring, mouth breathing	*Airway stability & circadian synchronization*
Q27	**Perceived Daily Stress Level**	Low / Moderate / High Burnout	*Sympathetic autonomic tone calibration*
Q28	**Alcohol, Smoking & Caffeine Intake**	Confidential clinical record	*Liver phase 1/2 detox & adrenal burden*

SECTION E — FOOD, NUTRITION & DIGESTIVE HABITS

Dietary structure, meal timings, gastrointestinal motility, and digestive symptoms.

Q#	Clinical Field / Question	Format / Options	Clinical Objective
Q29	**Dietary Preference Framework**	Vegetarian / Vegan / Eggetarian / Non-Veg / Jain	*Aligns root-cause food swaps to client values*
Q30	**Typical Daily Meal Timings**	Breakfast, Lunch, Dinner timestamps	*Assesses circadian fasting & insulin windows*
Q31	**Typical Day of Meals & Beverages**	Full food log from morning to night	*Evaluates macronutrient ratios & inflammatory triggers*
Q32	**Average Daily Water Intake**	Litres per day	*Hydration, kidney clearance & mucosal hydration*
Q33	**Digestive Symptoms in Detail**	Bloating, gas, reflux, acidity, bowel frequency	*Pinpoints upper vs lower GI dysbiosis*
Q34	**Foods Avoided or Disliked**	Open response	*Customizes palatable compliant meal plans*
Q35	**Eating Out / Delivery Frequency**	Times per week	*Industrial seed oil & sodium exposure assessment*
Q36	**Habit Obstacles & Difficulties**	Time crunch, travel, cravings, family meals	*Ensures sustainable realistic protocol adherence*

SECTION F — GOALS, EXPECTATIONS & DEDICATION

Define your desired outcomes so Reshmi Verma can engineer your targeted clinical protocol.

Q#	Clinical Field / Question	Format / Options	Clinical Objective
Q37	**Primary Outcome From Consultation**	Open response	*Anchor milestone for 1-on-1 healthcare roadmap*
Q38	**90-Day Transformation Vision**	What meaningful success looks like	*Defines tangible quality-of-life biomarkers*
Q39	**Past Nutrition Plans (What Worked/Failed)**	Open response	*Eliminates ineffective restrictive dieting habits*
Q40	**Preferred Style of Guidance**	Step-by-step plan / Breathwork routines / Biometrics	*Calibrates practitioner coaching delivery style*
Q41	**Daily Time Dedication Available**	Realistic minutes/day for breathwork & cooking	*Structures feasible non-overwhelming habit stacks*
Q42	**Specific Questions for Reshmi Verma**	Open response	*Prioritized agenda for clinical consultation call*

Reshmi Verma Clinical Philosophy:
"Resilience is not about pushing harder, but about creating the right foundation for the body to adapt, regulate, and recover."

Every assessment submitted through **healthwithreshmi.com** is reviewed with 18 years of diagnostic industry precision, integrating functional nutrition, Patrick McKeown's Oxygen Advantage respiratory biomechanics, and personalized 3R protocols.

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